Hand & Wrist Examination — OSCE Checklist

Deakin MD2 · MM6 · Time: 8–10 min

Tick each step as you perform or verbalise it. **Shaded rows (*)** *are optional or done if indicated.* **†** *= added from standard teaching (Talley & O'Connor), not in the Deakin guideline. Order:* **Look → Feel → Move (active, then passive) → Measure → Special tests.** *Includes* **Measure** *(function) for the hand.*

1. Introduction

☐	Wash hands; introduce self; confirm patient ID; explain and gain consent
☐	Position: seated on bed (or at a table), hands resting **palms down on a pillow**
☐	Exposure: hands and arms to the elbows
☐	Ask about pain before touching
☐	General observation: systemically well? in pain? age; other joint or skin disease (psoriasis); walking aids, splints

2. Examination

	Step	What to do / say	Abnormal
Look — forearms, wrists and hands (dorsal, then palms †)			
☐	Distribution	Symmetry, which joints are affected	Symmetrical small joints = RA; DIPs = OA, psoriatic
☐	Skin	Erythema, atrophy, rashes, scars (carpal tunnel or tendon release)	Psoriasis, previous surgery
☐	Nails	Pitting, onycholysis	Psoriatic arthritis
☐	Swelling	Joint swelling; "sausage" fingers	Synovitis; dactylitis (psoriatic)
☐	Deformity	Ulnar deviation, swan neck, boutonnière, Z-thumb; Heberden's/Bouchard's nodes †	RA; OA
☐	Wasting	Thenar or hypothenar eminence, dorsal interossei	Median / ulnar nerve lesion
☐	Function	Watch how they use their hands informally	
Feel			
☐	Temperature	Back of hand over wrists and joints	Warm = active inflammation
☐	Wrists	Palpate with both thumbs	Tenderness, synovitis
☐	MCP joints	Squeeze across MCPs gently, then palpate each †	Tenderness = synovitis (early RA)
☐	PIP and DIP joints	Palpate each joint	Boggy (synovitis) vs hard bony (OA)
☐	Joint features	Tenderness, synovitis/effusion, warmth, subluxation	
☐	Scaphoid*	After trauma: anatomical snuffbox and scaphoid tubercle (thenar)	Scaphoid fracture
☐	Pulses	Radial and ulnar pulses; capillary refill	Vascular compromise
Move — active			
☐	Wrist	Prayer and reverse prayer: flexion and extension †	Reduced range
☐	Fist and straighten	"Make a fist, then straighten your fingers"	Screen MCP/IP movement
☐	Thumb opposition	Touch thumb to each fingertip	
Move — passive			
☐	Wrists, MCP, PIP, DIP	Move each through range; feel for crepitus	Restriction, crepitus
Measure — hand function			
☐	Grip strength	"Squeeze my fingers"	Weak grip
☐	Key grip	Use lock and key to open lock	
☐	Opposition strength	Thumb to each finger, try to pull apart	Median nerve weakness
☐	Practical tasks	Undo screw-top jar; do up and undo a button; write with a pen	Functional impact
Special tests			
☐	Phalen's	Hold wrists fully flexed (backs of hands together) for 60 s †	Tingling in median distribution = carpal tunnel
☐	Tinel's	Tap over the flexor retinaculum (carpal tunnel)	Reproduces symptoms (less reliable)
☐	Neuro screen*	Sensory islands and key muscles of median, ulnar and radial nerves (covered in B&B)	Nerve injury — document sensation, movement, circulation after trauma

3. Closing

☐	Check the elbows: rheumatoid nodules, gouty tophi, psoriatic plaques
☐	Thank patient; wash hands
☐	Present: "There is no deformity, swelling or wasting, and the skin and nails are normal. No tenderness or synovitis. Full active and passive range of movement; grip, key grip and opposition are normal and function is preserved. Phalen's and Tinel's were negative."

4. Quick reference †

Feature	Rheumatoid arthritis	Osteoarthritis	Psoriatic arthritis
Joints	Symmetrical MCPs, PIPs, wrists; DIPs spared	DIPs, PIPs, thumb base (1st CMC)	DIPs, asymmetrical; dactylitis
Swelling	Soft, boggy synovitis	Hard, bony	Sausage digits
Deformities	Ulnar deviation, swan neck, boutonnière, Z-thumb, MCP subluxation	Heberden's (DIP) and Bouchard's (PIP) nodes, squaring of thumb base	Telescoping (arthritis mutilans)
Other	Rheumatoid nodules, wasting of small muscles	Crepitus	Nail pitting, onycholysis, plaques